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The
**Potty
Training**

PLAYBOOK



The research-backed guide to getting it right the first time

JACK HARTLEY

Dad of Two

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~ 45 minute read

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A note on medical advice: This book is based on published pediatric research and the author's experience applying it. It is not a substitute for guidance from your child's pediatrician. Every child is different. If something here does not match what you are seeing, trust your doctor over this guidebook.

Two Sons

FIRST SON & SECOND SON

My First Son is five years old. He is energetic, funny, and completely potty trained. Getting there was one of the hardest things we did as parents, and looking back, most of the hard parts were unnecessary.

My Second Son is three. He trained faster, smoother, and with less stress on everyone in the house. He is also energetic. Also funny. Roughly the same kid, temperamentally speaking.

The difference between those two experiences was not luck. It was not parenting talent. It was not which child was more cooperative or more ready. The difference was that by the time Second Son arrived, I had read every peer-reviewed study, clinical guideline, and parenting book I could find on potty training.

I am aware that most people do not deep-dive the research on a toddler milestone. Most people ask their pediatrician, get 90 seconds of advice, google a few things at midnight, and wing it. That is what I did with First Son. I winged it. I followed bad advice, started at the wrong time, used the wrong approach, and made just about every mistake in the book.

First Son is fine. Kids are resilient. He got there. But I spent a year and a half getting him there when it should have taken under ten months. And in that year and a half, there was stress, confusion, guilt, and more laundry than I care to remember.

When Second Son came along, I went looking for a resource that translated the research into plain language. Something that treated me like an adult who could handle real information. I couldn't find it. So I did the research myself. This book is what I put together.

THE SCIENCE

There is more rigorous research on potty training than most parents ever see. The American Academy of Pediatrics has formal guidelines. The Agency for Healthcare Research and Quality conducted a systematic review of all toilet training evidence. Multiple prospective studies in Pediatrics track outcomes based on when training starts, what methods are used, and how caregivers respond to setbacks.

None of that research is hard to find. It's just hard to find translated into something a parent can actually use at 7pm when everything is falling apart. That translation is what this book does. Each chapter gives you the real story from my house, what the research actually says in plain language, one clear lesson, and five things you can do right now.

THE PART NO ONE TALKS ABOUT

There is a version of potty training that no one posts about. The one where you're sitting on the bathroom floor at 6:47am, you've already changed the sheets once, your coffee is cold, and you genuinely cannot tell if your child is making progress or going backwards. You googled 'is my kid behind' at midnight and fell down a rabbit hole of milestone charts that made everything worse.

Parental stress during potty training is well documented, and it is circular. Parents who are stressed have children who have more accidents. Children who have more accidents produce more parental stress. The single most useful thing you can do for your child's progress is manage your own emotional state during the process. Not perform calm. Just find somewhere to put the frustration that isn't your child's face.

You are not failing. You are in the hard part. There is a difference.

THE LESSON

The difference between a hard potty training experience and a manageable one is rarely the child. It is almost always the information the parent had going in.

HOW TO USE THIS BOOK

Read it before you start training, not during a crisis. Read it with your partner. Both parents need to be aligned on language, timing, and responses. Inconsistency between caregivers is one of the most reliable ways to extend training. Don't skip Chapter 2. Timing is the highest-leverage decision in this whole process. Everything else builds on getting that one right.

Is Your Child Ready?

FIRST SON & SECOND SON

First Son was 19 months old when my wife pointed at him squatting in the corner of the living room and said: "He knows." She was right. He had that look. Total concentration, somewhere far away. And I thought: this is it. He's ready. We started training that week. Fourteen months later, he was done.

For over a year, we dealt with inconsistency, confusion, and a level of frustration I now know was almost entirely caused by one mistake. I started too early. With Second Son, I waited until 29 months. He checked 12 of 14 readiness signs. I cleared the calendar and started on a Monday. He was trained in eight months. Same house. Same parents. Ten months of difference. That is what getting the timing right is worth.

THE SCIENCE

The single most important decision in potty training isn't which method you use. It's when you start. A 2003 study in Pediatrics by Blum, Taubman, and Nemeth found that children who started intensive training before 27 months often took over a year to complete, while those who started between 27 and 32 months typically completed in under 10 months. Starting too late also creates challenges — delayed training is associated with increased incontinence issues.

The reason is neurological. Voluntary sphincter control develops between 24 and 30 months. Before that, a child can feel the urge but cannot reliably act on it in time. No method, no sticker chart, and no amount of patience overrides that timeline.

A readiness checklist also matters. A child who checks most of these boxes is ready to begin:

- > Keeps diaper dry for 90 minutes or more during the day
- > Shows awareness of going: pauses, squats, hides in a corner, goes quiet and still
- > Can pull pants up and down independently
- > Follows simple two-step instructions
- > Uses words or gestures to communicate needs
- > Shows interest in the toilet or in wearing underwear
- > Is bothered when the diaper is wet or soiled
- > Can walk to the potty and sit down without help
- > Has bowel movements on something of a schedule, not randomly at all hours
- > Understands basic bathroom vocabulary: wet, dry, going potty
- > Has started saying "me do it" or "by myself" about things generally

- > Can sit still in one spot for at least 2 minutes
- > Shows curiosity about what adults or older siblings do in the bathroom
- > Pauses briefly before an accident rather than going without warning

No child checks every box. If your child checks 10 or more, you're likely in the window. Fewer than 8, wait 4 to 6 weeks and check again. One more thing: girls are typically ready 2 to 3 months before boys. This isn't parenting. It's developmental biology. Boys who take longer aren't behind. They're on a different timeline.

THE LESSON

Starting at the right time is the highest-leverage decision in potty training. Get the window right and everything else gets easier. Get it wrong and no method in the world makes up for it.

TRY THIS

1. Use the checklist. If your child checks 10 or more, you're likely in the window. Fewer than 8, wait 4 to 6 more weeks and check again.
 2. Don't confuse awareness with readiness. A child who notices they're going is not the same as a child who can stop, walk to the potty, and get there in time. The first is instinct. The second is control.
 3. If you started early and it's taking forever, that's okay. It doesn't mean you failed. It means you started before the window. Keep going.
 4. Boys: add two months to your expectations. That's not a problem. That's normal.
 5. Mark your calendar. If your child is under 24 months, put a reminder 3 months from now to revisit the checklist. Don't force readiness. Wait for it.
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Before You Start

FIRST SON & SECOND SON

The week I started training First Son, three things happened that had no business happening during potty training week. My mother-in-law arrived for a five-day visit. My son picked up a runny nose from daycare. And we had a birthday party on Saturday requiring a two-hour car ride each way. I trained anyway. Life got in the way.

First Son was confused, overtired, off his routine, and surrounded by new energy. By Thursday I didn't know if we were making progress or going backwards. With Second Son, I spent two weeks preparing before day one. Equipment was ready. Every caregiver was aligned. I picked a stretch of five days with nothing on the calendar. The difference wasn't discipline. It was preparation.

THE SCIENCE

Research on toilet training consistently identifies consistency as one of the strongest predictors of success, more so than the specific method used. The Agency for Healthcare Research and Quality systematic review found that mixed signals between caregivers significantly extended training time regardless of approach.

Preparation falls into three areas. Equipment: the choice between a standalone potty chair and a toilet seat insert comes down to the child. Potty chairs are lower and allow kids to plant their feet, which matters for bowel movements. Either works. Caregivers: every adult in your child's life needs the same language, the same prompting schedule, the same response to accidents. Share your plan in writing. Timing: avoid starting during a new sibling, a house move, a change in childcare, or any significant family disruption.

THE LESSON

Preparation is not optional. Two weeks of setup before day one will save you two months of confusion after it.

TRY THIS

1. Buy the equipment this week, not the morning of day one. Get a potty chair or toilet insert, a step stool, and at least 10 pairs of underwear. Let them sit on the potty clothed. Make it ordinary before it becomes important.
2. Write a one-page plan and share it with every caregiver. Include: the words you use for pee and poop, how often you prompt, what you say at accidents, what you say at successes. One page. Hand it to daycare, grandparents, put it on the fridge.

3. Pick your window carefully. Find a stretch of at least three consecutive days, ideally five, with no major commitments and no visitors. That is your start date.
 4. Stock up on supplies: extra underwear, easy-on easy-off pants, a portable travel potty, a waterproof mattress cover. None of this is exciting to buy. All of it matters.
 5. Have the conversation the night before. Tell your child simply: "Tomorrow we start using the potty. Diapers are for sleeping. You're ready." Keep it calm, short, positive.
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The Method

FIRST SON & SECOND SON

With First Son, I didn't really have a method. I had a vague plan to put him on the potty sometimes, keep diapers available just in case, and hope it resolved itself over time. It did not resolve itself. He spent weeks in an in-between state: sometimes the potty, sometimes a diaper, always confused about which was expected. I thought the problem was him. It wasn't. The problem was that I had given him no clear signal about what was expected.

With Second Son, I followed the intensive method from day one. Naked from the waist down for the first two days. Then pants with no underwear. Then underwear. No diapers except for sleep. No asking if he needed to go. Within a week, the pattern was set. He knew what was expected. The uncertainty was gone.

THE SCIENCE

The intensive method was originally developed by Azrin and Foxx in 1974 as a single-day clinical program. A 1976 study by Butler documented success rates above 90% in the original protocol. That context matters: the original program was administered with trained support in a controlled setting. The home-based, multi-day adaptation most families use today is a different application of the same principles, and real-world outcomes vary depending on readiness, consistency, and the conditions at home. That caveat does not change the basic argument. The underlying behavioral approach has more evidence behind it than anything else in the literature.

The method works in four progressive stages: (1) Naked from the waist down -- no clothing means no interference between sensation and response. (2) Commando -- pants on, no underwear. The child begins to generalize the skill. (3) Underwear added -- full clothing, the child is essentially trained. (4) Outside the home -- public restrooms, parks, restaurants. See Chapter 8.

Two rules apply through every stage. First: don't ask, tell. "Do you need to go?" gives a toddler an easy no. Say instead: "Let's go sit on the potty." It's not a question. Second: prompt on a schedule. Take your child to the potty every 90 minutes regardless of signals. In the early stages, they cannot reliably read their own urgency in time.

THE LESSON

A clear method, applied consistently, beats any amount of patient hoping. Every time you reach for the diaper during training hours, you're not being kind. You're changing the rules.

TRY THIS

1. Commit to the stages in order. Naked first, then commando, then underwear. Each stage consolidates the one before it. Don't skip ahead because one day looks good.
 2. Set a timer for 90 minutes and use it. When it goes off, you go to the potty. Not in a few minutes. Now. The schedule is the method.
 3. Switch to "let's go" instead of "do you need to go?" tonight. Start building the language habit now.
 4. No diapers during waking hours once training starts. Not for outings, not for errands. The diaper sends a message that the rules are negotiable. They are not.
 5. Track the first three days. Write down the time of every accident and every success. You will see a pattern. That data makes the second week easier.
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CHAPTER 5

The First Three Days

FIRST SON & SECOND SON

Day two with First Son, I cracked. We'd had a reasonable day one. Then day two: seven accidents before noon. I told myself we needed a reset and put him back in a diaper for the afternoon. That was the moment the whole thing fell apart. The diaper said: the rules have conditions. First Son learned that lesson clearly. We spent the next several months unlearning it.

With Second Son, day two was also hard. There were accidents, there was resistance. I stayed calm, kept the timer running, and did not reach for the diapers. By day three, something had shifted. He was starting to move toward the potty on his own before I prompted him. The first three days are where most parents quit. They are also where the whole thing is decided.

THE SCIENCE

Studies on the intensive method show that the first three days are simultaneously the highest-accident period and the highest-learning period. Children pulled back to diapers mid-process don't simply resume where they left off. They learn the transition is reversible, which adds weeks to overall training time.

The emotional arc is predictable and worth knowing in advance. Day one is often surprisingly good -- novelty produces early successes. Day two is typically the hardest: the novelty has worn off, accidents spike, and this is the day most parents lose faith. It is also the day holding the line matters most. Day three usually shows a visible shift. Not perfection -- but the child begins to anticipate rather than react.

Research shows that parental emotional state directly influences child behavior during this period. Parents who remain calm during accidents have children who recover faster. This is not about pretending everything is fine. It is about not letting your frustration become your child's stress.

THE LESSON

The first three days are the hardest and the most important. Don't mistake day two difficulty for failure. It's part of the process.

TRY THIS

1. "Day two is supposed to be hard." Write that somewhere visible before you start. You will need to read it on day two.
 2. Plan your own schedule for the first three days as carefully as your child's. Clear your obligations, have easy food ready, build in moments to reset emotionally. You cannot stay calm if you are also managing three other urgent things.
 3. Celebrate every success, however small. "You felt it and you made it. That's it." Keep it real, not theatrical.
 4. "Your body had an accident. Let's clean up." Same words, neutral tone, every time. Then move on immediately.
 5. Do not return to diapers during waking hours. What looks like failure on day two is almost always just the natural low point of the process. Wait until day three before drawing any conclusions.
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The Poop Chapter

FIRST SON & SECOND SON

First Son was pee-trained within two weeks. Then came the standoff. For 19 days, he refused to poop on the potty. I pushed. I bribed. I reasoned with a two-year-old about the logic of using the toilet. None of it worked, and some of it made things worse. Eventually it resolved -- not because of anything I did, but because he was ready and I had finally stopped making it a battle.

With Second Son, I was ready for this. When I saw the first signs of poop resistance, I immediately removed all pressure. I let him ask for a diaper for bowel movements while keeping all pee training intact. I said nothing about it. Four days later he pooped on the potty on his own. 19 days versus 4. The difference was entirely in how I responded.

THE SCIENCE

Stool toileting refusal occurs in approximately 20% of children during training. One in five. It is not a sign of a problem child or a failing parent. It is a predictable developmental event. The cause is usually a combination of two things: the bowel has always been the child's private domain, and if the child has experienced any constipation or discomfort, they may associate the potty with pain and begin withholding as a protective response.

Withholding makes constipation worse. This cycle, if left unaddressed, can lead to encopresis -- involuntary leakage caused by chronic impaction, affecting 1 to 4% of school-age children. It is almost entirely preventable. The clinical guidance is consistent: remove all pressure from bowel training, maintain pee training, and allow the child to use a diaper for bowel movements temporarily if needed. A 2003 study by Taubman, Blum, and Nemeth found that positive parental language shortened how long it lasted. Negative language, frustration, disgust -- made it last longer.

Watch for constipation: hard stools, going fewer than three times a week, avoiding the bathroom, hiding in corners. Address the constipation before addressing the refusal. For toddlers, polyethylene glycol (sold as MiraLax) is tasteless, mixes into any liquid, and has a strong pediatric safety record. Confirm the dose with your pediatrician before starting.

THE LESSON

Poop takes longer than pee. This is normal. The only thing that reliably makes it take longer is pressure. Remove the pressure, and most cases resolve on their own.

TRY THIS

1. Separate poop from pee in your mind and in your approach. These are two different training tasks with different timelines. Celebrate pee success. Say nothing about poop.
 2. If your child asks for a diaper to poop, give it to them. Without comment. Without a face. This is a temporary step, not a defeat.
 3. Watch for constipation signals: straining, hard pellet stools, going fewer than three times a week. Address the constipation before addressing the refusal.
 4. Make the bathroom comfortable. A warm seat, a step stool for foot grounding, letting the child control the flush. Small changes that reduce resistance.
 5. Give it time. Most stool refusal resolves within four to eight weeks when pressure is removed. Still unresolved after eight weeks, or constipation is involved? Talk to your pediatrician.
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CHAPTER 7

When It Falls Apart

FIRST SON & SECOND SON

First Son was trained at 33 months. Solid. Consistent. Then his baby brother arrived. Within a week, First Son had regressed completely. I was exhausted with a newborn, I had no patience, and I made the mistake of showing it. He regressed further. It took six weeks to get back to where we were. It should have taken two.

When Second Son had his own regression six months into training, triggered by a new daycare setting, I handled it differently. I acknowledged the change, stayed neutral about accidents, went back to more frequent prompting, and said nothing about the regression itself. Two weeks later it was over. My response determined how long the recovery took.

THE SCIENCE

Regression after successful training is common, normal, and well-documented. The AAP identifies the most frequent triggers: a new sibling, a move, a change in childcare, illness, and general family stress. Regression is not failure. It is a sign that the child is under stress and reverting to a more familiar behavior pattern.

Around age three, children develop the capacity for shame. They may start hiding accidents or cleaning up before a parent notices. This is actually a good sign. It means they know what they're supposed to do. Don't treat it as deception. Meet it with calm acknowledgment: 'You had an accident. That's okay. Let's clean up.'

One note that belongs in a different category: if your child has been fully dry for six months or more and suddenly begins wetting again with no obvious cause, do not wait two weeks. See your pediatrician. It could be a UTI, or occasionally something else your doctor should check. What this chapter describes is behavioral regression triggered by stress. A child who was dry for six months and then wasn't, for no clear reason, needs a medical check, not a training reset.

THE LESSON

Regression is not failure. It is information. Something in your child's world has changed, and their body is telling you before their words can. Respond to the cause, not the behavior.

TRY THIS

1. When regression hits, identify the trigger first. Name it out loud with your child: "It must be a big adjustment having a new baby at home." Kids don't need solutions. They need to feel seen.
 2. Return to basics without drama. More frequent prompts, lighter clothing, easier bathroom access. Do it quietly. Don't announce you're going back to basics.
 3. Do not return to diapers full-time. Training pants under underwear is a temporary compromise. Maintain the standard. The expectation hasn't changed.
 4. For strong-willed children, remove the power struggle. Replace "let's go to the potty" with "do you want to walk to the potty or hop?" Both options end at the potty. The choice is enough.
 5. Give it two weeks before escalating concern. Most regressions, handled calmly, resolve within two weeks. After three weeks, or if emotional distress persists, mention it to your pediatrician.
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Leaving the House

FIRST SON & SECOND SON

I kept First Son in diapers for all outings for the first six weeks of training. I told myself it was practical. What I was actually doing was avoiding the anxiety of a public accident. The result was that First Son learned the rules had an exception: out of the house, the old way came back. It took months to undo that lesson.

With Second Son, we left the house diaper-free on day four. I packed backup clothes, brought a portable travel potty, and found the bathroom the moment we arrived anywhere. We had one accident in a parking lot. He found it more funny than distressing. Day four felt reckless. It wasn't. By week two, going out was as routine as staying in.

THE SCIENCE

Here is something worth knowing about how kids learn: a skill your child picks up at home does not automatically transfer to new places. The home bathroom and a park restroom are completely different experiences for a two-year-old. Research shows that children trained exclusively at home take significantly longer to achieve consistent dryness than those who practice in varied environments from early in the process.

The automatic flush sensor is the most common public restroom fear. Covering it with a sticky note before your child sits down eliminates this almost entirely -- it startles a lot of kids and can put them off the toilet for weeks. Daycare is its own challenge: if daycare is still using pull-ups after you've moved to underwear at home, the mixed message will slow things down. Most daycares will follow a written training plan if you provide one.

THE LESSON

The real world is part of the training. A child who is trained only at home is not fully trained. Get out early and often.

TRY THIS

1. Go out on day four or five, not day fourteen. Bring backup clothes, a travel potty, and low expectations. The goal is exposure, not a perfect trip.
2. Find the bathroom first, every time you arrive anywhere new. Before the park, before the restaurant. Walk your child through it. Make it routine.

3. Cover the automatic flush sensor with a sticky note before your child sits down. Keep a small supply in your bag. This costs nothing and prevents one of the most common fears.
 4. Visit daycare or school before training begins. Walk through the bathroom with your child. Let them see the toilets, flush them voluntarily, and feel comfortable before there is any pressure.
 5. Pack the same things every time for the first month: two full outfit changes, portable travel potty, wipes, plastic bags for soiled clothing. The bag becomes a habit.
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Nighttime

FIRST SON & SECOND SON

I put First Son in underwear at night at 34 months because he had been daytime trained for a month. He wet the bed every night for a week. I woke him at midnight. I restricted fluids after 5pm. I got frustrated. None of it helped. I was trying to train something that cannot be trained.

With Second Son, I waited. He stayed in pull-ups at night until he was consistently waking up dry, which happened around 38 months. I did nothing to accelerate it. I just watched for the pattern and acted when it was there. Zero bed-wetting episodes after the transition. The difference was understanding what nighttime dryness actually is.

THE SCIENCE

Nighttime bladder control is neurological, not behavioral. It is driven by the development of vasopressin, an antidiuretic hormone that suppresses urine production during sleep. Until this hormone is functioning reliably overnight, the child's body will produce urine at a rate the bladder cannot hold. This development cannot be rushed -- not by instruction, not by reward systems, not by restricting fluids.

The data on bedwetting: approximately 30% of children are still wetting the bed at age four and a half. Around 15 to 20% at age five. Up to 10% at age seven. About 5% at age ten. All within the normal developmental range. Bedwetting is not a medical concern until age seven. When it persists, bedwetting alarms have 50 to 80% success rates across randomized trials and produce more lasting results than medication.

THE LESSON

Nighttime dryness is biological, not behavioral. You cannot train it. You can only recognize when it has arrived and remove the pull-up at that point.

TRY THIS

1. Keep the pull-up at night until your child is waking dry consistently -- 5 of 7 mornings for two to three weeks in a row. That is your signal.
2. Do not restrict fluids in the evening. It does not prevent bedwetting. Hydration matters for healthy bladder development and for sleep.
3. Don't wake your child for a midnight bathroom trip. It disrupts sleep without meaningfully reducing bedwetting.

4. When you make the transition, put a waterproof mattress cover on before day one. Not after the first accident. Before.
 5. If your child is still wetting the bed at age seven, talk to your pediatrician about a bedwetting alarm. It works for the majority of children and the results last after the alarm is discontinued.
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Boys and Girls

FIRST SON & SECOND SON

My sister-in-law called me when her first daughter was 22 months old. She had started training two weeks earlier. Things were not going well. Same pattern I recognized from First Son: too early, no clear method, asking instead of telling, diapers still available during outings. I walked her through the research. She waited two more months. Her daughter trained at 27 months in under nine months. Her second daughter, trained from the start with the right timing and method, was done at 30 months in seven. Both faster than First Son. Both smoother.

THE SCIENCE

Research consistently shows that girls achieve potty training readiness approximately 2 to 3 months earlier than boys and complete training 2 to 3 months faster. A study by Schum and colleagues in Pediatrics tracked 267 children and found that girls reached daytime dryness at a median age of 32.5 months, compared to 35 months for boys. Girls tend to develop language and fine motor skills earlier -- both of which support the communication and physical aspects of training.

For boys: the AAP recommends starting seated for both urination and defecation during initial training. Standing is a secondary skill to introduce only after seated training is reliable. For girls: front-to-back wiping is an important hygiene habit to establish from day one. Wiping back to front can introduce bacteria into the urethra, causing UTIs, which are more common in girls and which complicate the training period if they occur. If your daughter develops sudden urgency, burning, or frequent accidents after a period of good progress, consider a UTI before assuming behavioral regression.

THE LESSON

Boys and girls are on different timelines. That is not a problem to fix. It is a fact to plan around.

TRY THIS

1. For boys: start seated. Introduce standing urination only after sitting is consistent and your son expresses interest. There is no rush.
2. For girls: teach front to back wiping from day one. "Wipe from front to back." Same words, every time, until it is automatic.

3. For boys training later than expected: if your son is 32 months and not trained, that is not late. That is average. If he is 38 months and still struggling, talk to your pediatrician.
 4. For girls training earlier than expected: readiness signs still matter more than age. Check the checklist in Chapter 2, not the calendar.
 5. For families with both: do not compare their timelines. The girl will likely go faster. That says nothing about the boy's capability.
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Done

FIRST SON & SECOND SON

There was no single moment with either son when I thought: we are done. With First Son, there was just a gradual thinning of accidents, followed by a long stretch without one, followed by the realization somewhere around month sixteen that I had stopped thinking about it. With Second Son, the process was shorter, but the ending was similarly quiet. Nobody announced it. That's how it ends. Not with a ceremony. Just the absence of the thing that had been consuming your attention.

THE SCIENCE

Clinically, a child is considered trained when they're consistently dry during the day, using the toilet on their own, handling their own clothes, for at least two weeks without accidents. Most children reach this point between 30 and 48 months for daytime training. Nighttime dryness is a separate milestone on its own timeline.

Talk to your pediatrician if: your child is past age five with regular daytime accidents despite consistent effort; your child suddenly regresses after a long dry period with no identifiable stressor; your child experiences pain during urination or defecation; or you see unusual patterns in frequency or urgency. If none of those apply, the range of normal is wide. Some kids are done at 28 months. Some are still working on it at 42. Both are fine.

THE LESSON

Done is not a moment. It's a pattern. When the accidents have stopped and the routine is independent and the potty is just where you go, you're done. The habit belongs to them now.

TRY THIS

1. Two weeks of accident-free, independent bathroom use is your finish line. Not one good day. Two weeks. Then you're done.
2. Keep a waterproof mattress cover on through the first year of nighttime dryness. Occasional accidents during illness are normal.
3. Don't announce that training is over to your child. Just let the sticker chart disappear and the topic stop coming up. They don't need closure ceremonies.

4. Take note of what worked for your child specifically. Write it down. When the next child comes along, you'll have your own story to draw from.
 5. Give yourself some credit. You read the research, you prepared, you stayed calm when it was hard, and you got through it. That's the job. You did it.
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BONUS

The Rewards System

FIRST SON & SECOND SON

With First Son, I avoided rewards. I had read somewhere that external rewards undermined intrinsic motivation, and I didn't want to raise a child who needed a sticker to do basic things. First Son had very little motivation to use the potty. In the absence of any external incentive and without sufficient internal motivation yet developed, sitting on a cold plastic chair to do something unfamiliar had no appeal.

With Second Son, I used a sticker chart from day one and a small food treat for every successful trip. I felt slightly guilty about it. I faded both over about ten days as the habit solidified. By day twelve, he didn't ask about the sticker chart and I had quietly put it away. He is now fully trained and has no particular attachment to small treats as a life requirement. The reward system did exactly what it was supposed to do.

THE SCIENCE

Positive reinforcement is among the most well-documented principles in behavioral psychology. The concern about undermining intrinsic motivation -- the overjustification effect -- applies primarily to activities the child already finds intrinsically interesting. Sitting on a potty at age two is not one of those activities.

Research on potty training shows that tangible rewards meaningfully accelerate acquisition of the target behavior. The critical factor is fading the reward intentionally once the behavior is established. The fading process: reward every success for days 1 to 5; reward only the first success per day after that; then intermittently; by two weeks most children have internalized the behavior and the reward is unnecessary. Specific praise -- 'you felt the urge and got there in time' -- is more effective than general praise and requires no fading.

THE LESSON

Rewards are a tool, not a bribe. Use them intentionally during acquisition, fade them deliberately as the habit forms, and don't feel guilty about either.

TRY THIS

1. Start with a sticker chart on day one. One sticker per successful potty trip on a chart on the bathroom wall. The child puts the sticker on themselves. The act of placing it is part of the reward.

2. Add a small food treat for the first five days if motivation seems low. Something small your child genuinely values -- a raisin, a Cheerio, a single piece of their favorite treat. One piece, immediately after success, every time.
 3. Begin fading the food treat on day six. One per day instead of one per trip. Then every other day. By day ten, it is gone. If your child asks about it, acknowledge it simply and move on.
 4. Keep the sticker chart up for two to three weeks, then quietly retire it. No announcement needed. Just stop replacing the stickers.
 5. Use specific praise throughout and never fade it. "You felt it coming and made it to the potty. That's the whole thing." This costs nothing, creates no dependency, and remains effective indefinitely.
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ABOUT THE AUTHOR

Jack Hartley is a dad of two. First Son taught him what not to do. Second Son gave him a chance to get it right. Between the two, he reviewed more than forty peer-reviewed studies and clinical guidelines from the American Academy of Pediatrics. He is not a pediatrician. He is a parent who did the reading so you don't have to.

The Potty Training Playbook is the first in a series of research-backed guides to the milestones that matter.

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